Pivotum

THE DEGREE RISK INDEX · FALL 2026 EDITION

Is Nursing Safe From AI?

The complete profile — the full scores by track, the six factors behind them, the honest downsides, and what to ask before you commit.

2.8

AI EXPOSURE · BEDSIDE RN (HOSPITAL, ACUTE CARE)

where 10 is most at risk

Read this one first; send the student version to them.

HIGHLIGHTING IS COLOUR-CODED

THE FINDING EXPOSURE PROTECTION METHOD & HONESTY

Before you read this — a note for parents

You're likely reading this before your child is. That's normal, and it's worth pausing on how you use it.

Don't lead with the scores. If you open the conversation with a risk number, one of two things happens: they get frightened and shut down, or they get defensive about a decision they've already made. Neither produces a better choice.

The better approach: read this yourself first, then share it with them as *information you both get to react to*, not as a verdict you've reached. The discussion questions at the end are designed to be worked through together — they answer, you answer, you compare. Some of them have no right answer at all.

What this is. This is a facts document, not a recommendation. We've set out what's known, where the evidence is contested, where we might be wrong, and what the profession itself says — including the parts that don't support our own conclusions. What it deliberately doesn't do is tell your child what to choose. Nobody who hasn't met them is in a position to.

Three things to keep in mind:

1. **They're choosing, not you.** Your job here is to widen what they've considered, not narrow it to your preference. 2. **This is a starting point, not a ruling.** A score is a prompt for a conversation. "This is interesting — want to look at why?" works. "This says do nursing" doesn't. 3. **Don't do it all in one sitting.** The best versions of these conversations happen over weeks, in cars and on walks, not across a kitchen table with a printout.

One more thing: if they're already certain about nursing, that's an asset, not an obstacle. Commitment is worth a great deal. Don't try to unpick it — use this to help them build the strongest possible version of the path they've chosen.

A practical note: there's a separate short version written directly to them rather than about them — it should have come alongside this document. Realistically, a seventeen-year-old may not read fourteen sections, and their engagement is the whole point. Send them that one. This document is for you.

The short answer

Yes — nursing is one of the strongest degree bets available, protected by hands, trust, and law. But the protection lives at the bedside, not in the badge: the desk-based roles the same degree leads to score nearly twice as high.

That distinction is the single most useful thing in this profile, and it's the thing a quick internet search will never tell you.

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1. The scores

TRACK	2023	2025	FALL 2026	3-YR MOVEMENT	BAND
Bedside RN (hospital, acute care)	2.1	2.4	**2.8**	+0.7	Low
Community / home health nursing	2.2	2.4	**2.7**	+0.5	Low
Specialist clinical (ICU, ED, peri-op)	1.9	2.1	**2.4**	+0.5	Low
Nurse practitioner / advanced practice	2.6	2.9	**3.2**	+0.6	Low
Telehealth triage	4.1	4.8	**5.2**	+1.1	Moderate
Utilization review / case management (desk)	4.3	5.0	**5.4**	+1.1	Moderate

Scores run 1–10, where 10 is most at risk. For context: the median profession in this edition sits around 5.5, and entry-level software development scores 8.1.

Read the table this way: every clinical, in-person track sits between 2.4 and 3.2 — the lowest band in the entire index. Every desk-based track sits above 5. Same license, same degree, roughly double the exposure.

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2. Why nursing scores low — the six factors

How much of this job can AI already do?	5.5
How hard will it be to land that first job?	2.0
Does it have to be done in person, with your hands?	9.0
Does someone need a human they can trust and hold responsible?	9.0
Does the law require a licensed human?	9.0
How often does the job hit genuinely new, high-stakes situations?	8.0

EXPOSURE RAISES THE SCORE PROTECTION LOWERS IT

We score every profession in this index against the same six questions, with the same weights. Here's how nursing answers them.

HOW MUCH OF THIS JOB CAN AI ALREADY DO? (35% OF THE SCORE)

Moderate — and concentrated entirely in the paperwork.

More of nursing is automating than most people assume: charting, documentation, shift handover summaries, care-plan drafting, triage protocols. Ambient documentation tools — systems that listen to a nurse-patient interaction and generate the structured record automatically — are already deployed in hospitals and being trialled formally.

What is *not* automating is the core: physical assessment, hands-on care, and noticing that a patient is deteriorating before the monitor says so. Clinical intuition built from thousands of hours at bedsides is not a documentation task.

One useful technical detail: nursing documentation is structurally different from physician documentation. A doctor writes one or two narrative notes a day; a nurse records hundreds of structured observations across a shift. Tools built for physician workflows don't simply transfer, which is part of why nursing's automation has lagged medicine's.

HOW HARD WILL IT BE TO LAND THAT FIRST JOB? (15%)

Easier than almost any field we score — and for a structural reason worth understanding properly.

This is the most under-appreciated advantage of a nursing degree, and it's the factor almost no career advice accounts for.

Across most professions, AI is eating the *junior* tasks first — the grunt work that entry-level roles were built from, and that people historically used to build judgment. Junior developers, paralegals, entry analysts, production designers: their on-ramps are narrowing badly. The professions survive; the paths into them are collapsing.

Nursing is nearly alone in being immune to this, because clinical hours are mandated by licensure. A nursing student cannot graduate without supervised real-world practice. The on-ramp is written into law, so it cannot quietly erode the way a market-driven one can. Combine that with persistent nursing shortages, and a new graduate enters one of the more welcoming job markets in this index.

DOES IT HAVE TO BE DONE IN PERSON, WITH YOUR HANDS? (15%)

Overwhelmingly yes — the strongest physical protection we score.

Turning a patient. Starting a line. Reading a body in a room. Robotics is nowhere near this, and the gap is measured in decades rather than product cycles. Bedside nursing happens in unstructured, unpredictable environments with a human body at the centre — the single hardest problem in robotics, not the easiest.

DOES SOMEONE NEED A HUMAN THEY CAN TRUST AND HOLD RESPONSIBLE? (15%)

Absolutely — this is nursing's deepest protection.

Nursing is trust work as much as clinical work: frightened patients, difficult families, end-of-life conversations, the person who explains what the doctor just said after the doctor leaves. And someone must be accountable when a clinical decision goes wrong — legally, professionally, and morally.

DOES THE LAW REQUIRE A LICENSED HUMAN? (10%)

Yes, comprehensively.

Licensure, scope-of-practice rules, and staffing ratios all mandate human nurses. This matters more than it appears: regulatory protection moves at the speed of legislation and public trust, not at the speed of technology. It's the most durable protection in our entire framework, because AI capability alone cannot repeal it.

HOW OFTEN DOES THE JOB HIT GENUINELY NEW, HIGH-STAKES SITUATIONS? (10%)

Constantly.

Patients present atypically as a rule. They deteriorate unpredictably, arrive with several conditions interacting, and don't match the textbook. This is precisely where AI remains weakest — genuine novelty, where the pattern doesn't fit and being wrong is costly.

3. What the trend shows

We retrospectively scored nursing for 2023 and 2025 using this same methodology. It did not come out flat — and that's the honest result.

Bedside RN: 2.1 → 2.4 → 2.8. Up 0.7 points in three years. Real movement, but slow. And every point of it traces to one factor: documentation and administrative automation. The physical, trust, licensing and judgment scores have not moved *at all* across three years.

For comparison over the same period, an entry-level software developer moved 1.8 points. Nursing is drifting. Software is sprinting.

But look at the second group. Telehealth triage and utilization review — desk-based roles a nursing degree leads to — moved 4.1 → 5.2 and 4.3 → 5.4. Faster than bedside, and from a much higher base. Strip away the physical presence and in-person trust, and most of nursing's protection goes with it. What's left is a screen, a protocol, and a decision — which is what AI is good at.

The finding that matters: nursing's shield is the bedside, not the badge.

4. How durable is the protection?

Every "AI can't do X" boundary has proven temporary. It couldn't do nuance, then it could. Couldn't do images, then it could. So the useful question isn't whether nursing's protections hold today — they clearly do — but how long each is likely to last.

PROTECTION	STRENGTH TODAY	DURABILITY	WHERE THE PRESSURE IS COMING FROM
Regulatory / licensing	Strong	**Most durable**	Moves at the speed of law and public trust. AI capability doesn't repeal a licensing requirement or a staffing ratio.
Human trust / accountability	Strong	**Durable**	Socially rooted. People want a human present in vulnerable moments, and someone must own the outcome. Erodes slowly, if at all.
Judgment under novelty	Strong	**Eroding slowly**	Clinical decision support and triage algorithms are advancing — currently assisting, not deciding.
Physical / embodied	Strongest	**Most fragile long-term**	The one protection physical AI directly targets. Strongest shield today; potentially the weakest in twenty years.

The honest read for a seventeen-year-old: they will practice for roughly forty years. Over that horizon, physical protection is the one to watch — but bedside care sits near the *back* of the robotics queue, well behind warehouse work, assembly lines and structured manual tasks, because it's unpredictable, unstructured and trust-dependent. Meanwhile their two most durable protections, licensure and human trust, are the ones least affected by any technical advance.

5. How the role will actually change

Not "will nursing exist" — it will. The useful question is what the job looks like in ten years.

WAS THE JOB	IS BECOMING THE JOB
Hours of charting per shift	Ambient systems draft the record; the nurse verifies and corrects
Manual observation logging	Continuous monitoring flags changes; the nurse interprets them
Protocol recall from memory	Decision support surfaces the protocol; the nurse judges whether it fits *this* patient
Documentation squeezing out patient time	More time at the bedside — the part most nurses came for

The unusual thing about nursing is not the direction of automation — it's what gets automated. AI takes the routine, repeatable layer first in every profession we score; that's consistent. What differs is whether that layer was the part practitioners valued. In creative and knowledge work it often was — the designer's design, the writer's draft — leaving people to review output rather than produce it. In nursing the routine layer is documentation, which is the part that drives nurses out. Automating it returns time to direct care rather than removing the craft.

That's a genuinely positive story, and one worth saying to a prospective student clearly. The main caveats: verification of AI-generated records becomes a real clinical responsibility (a wrong note is a patient-safety issue), and the volume of monitoring data can create alert fatigue rather than relief if implemented badly.

6. Human strengths that will matter most

Ranked for nursing specifically, by how well each resists the coming decade:

1. **Physical assessment and clinical intuition** — noticing the patient who "looks wrong" before the numbers say so. The hardest thing to replicate and the most valuable thing they'll build. 2. **Being trusted in a vulnerable moment** — the human in the room when someone is frightened. Durable, socially rooted, and unautomatable in any near horizon. 3. **Clinical judgment under ambiguity** — the atypical presentation, the interacting conditions, the case that doesn't match anything. 4. **Verification and discernment** — knowing when an AI-generated note, alert or triage recommendation is subtly wrong. This is a *new* core skill and it is barely taught anywhere yet. A graduate who has it will stand out immediately. 5. **Communication and advocacy** — with families, with doctors, across a care team. Increasingly the differentiator as documentation ceases to be a differentiator at all.

Notably *not* on this list: speed at charting and volume of documentation. These were markers of a strong new nurse and are the fastest-depreciating.

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7. Other things to consider about this job

Everything so far has measured one thing: how exposed nursing is to AI. That's useful, and it's deliberately narrow. It says nothing about whether the work is rewarding, whether it's hard, whether it suits a particular person, or whether the job market is short of people or drowning in them.

All of that matters more to a real decision than the score does. So here it is, in four parts.

What's genuinely good about it

WHAT NURSES THEMSELVES SAY

The satisfaction data is more positive than the burnout headlines suggest, and the gap between the two is the most interesting thing in it:

- 68% of nurses rate their decision to join the profession a 4 or 5 out of 5.
- **81% say their nursing education was worth it.**
- The survey's own summary of a difficult year: *nurses aren't giving up on nursing — their love for the profession is as strong as ever*. What's under strain is the conditions, not the calling.

One honest counterpoint, because it belongs here: only 47% say they'd recommend nursing to a friend or family member. That gap — most would choose it again themselves, fewer would recommend it to someone they love — is the whole tension of this profession in a single statistic. It's worth sitting with rather than explaining away.

WHAT PEOPLE FIND REWARDING ABOUT IT

When nurses are asked what's most rewarding, the answers come back in a consistent order:

1. **Making a difference in people's lives** — the most-cited answer every year it's asked. 2. **Gratitude and relationships with patients** — being trusted at the hardest moments of someone's life. 3. **Pride in the work itself** — in being a nurse, and in the standard of care they give. 4. **Being good at something difficult** — competence in a demanding craft. 5. **The team** — nursing is unusually collegial; the bond between people who've handled a bad shift together is

real and frequently cited. 6. **Variety of settings** — few qualifications let someone work in this many environments, countries, and specialties.

Notice what's on that list: **meaning, relationship, mastery and belonging.** Those are the things careers research consistently identifies as producing durable job satisfaction — and nursing delivers all four when the conditions allow it. That's rare, and it's why people stay in it despite everything in section 7.

SETTING MATTERS ENORMOUSLY — AND THIS IS ACTIONABLE

Satisfaction varies more *within* nursing than between nursing and other careers. The 2026 State of Nursing Survey found nurse educators reporting around 68% satisfaction, with NICU and hospice also rating highly — driven by greater autonomy and far lower exposure to workplace violence. At the other end, progressive care sat near 23%, with geriatric at 31%, telemetry at 34% and emergency at 35%, driven by unsafe staffing ratios and higher exposure to abuse. Over half of emergency nurses reported being assaulted at work.

And here is the finding that should shape the decision. Medscape's survey of 7,723 practising nurses found that 73% would choose nursing again — but only 45% would choose the same specialty. Nurses are far more likely to regret where they landed than to regret the profession itself.

That's a 45-point spread inside the same profession. It means "should I be a nurse?" is a much less useful question than "what kind of nurse, where?" — and it's a strong argument for a program with broad clinical placements, so they find out which environments suit them before committing.

THE PRACTICAL CASE

- **Genuine job security.** Not "probably fine" — a projected shortage of over 250,000 RNs by 2028, with roughly 190,000 openings a year. Very few graduates in this index enter a market actively short of them.
- **A portable qualification.** It works in every state and most countries. Few degrees travel like this.
- **Fast to earning.** Two years via an ADN, four via a BSN — against median nursing pay around $80,000 and considerably more with advanced practice or specialization.
- **A ladder that's actually climbable.** Specialization, advanced practice, education, informatics, leadership — with employers frequently funding the study, which is unusual.
- **Flexibility over a lifetime.** Full-time, part-time, agency, travel, per diem. It flexes around a life in ways most professions don't.

What's genuinely hard about it

For a seventeen-year-old choosing nursing, automation is not the biggest risk to this career. Leaving it is. We'd be selling you something dishonest if we spent ten pages on AI exposure and said nothing about the thing that actually ends most nursing careers.

THE NUMBERS

Turnover is high, and it's concentrated at the start. The NSI National Health Care Retention Report — the most comprehensive annual survey in the field, covering 527 hospitals across 40 states and 262,405 registered nurses — put national RN turnover at **17.6% in 2025**, up 1.2 points and reversing several years of improvement.

The early-career concentration is stark: 22.7% of newly hired RNs left within their first year, and 56.8% of all departing staff had less than two years of service. Replacing one bedside RN costs a hospital around $60,090.

One nuance worth knowing, because it cuts against the simple version. Retention among the youngest nurses is actually *strong* during the first two years, where residency programmes and structured onboarding hold people in place. Turnover rises after roughly the thirty-month mark — when the support scaffolding comes off. That has a practical implication: the quality of a first employer's residency programme matters enormously, and so does what happens when it ends.

Intention to leave is widespread, and it worsened this year. The 2026 State of Nursing Survey, covering 2,090 nurses, found job satisfaction fell to **47%, down from 55%** the previous year — though still far above the 2022 low of 28%.

43% said they were likely to leave the bedside within a year, up from 38%. The share saying they'd likely leave nursing entirely jumped from 15% to 23% — close to a 50% increase in a single year.

The most revealing figure in that survey isn't any of those. Asked why they stay at the bedside despite wanting to leave, 41% of nurses cited financial necessity — more than four times the number who cited good management or leadership support (8%). That is the honest state of the profession's working conditions, and it has nothing to do with AI.

But — and this matters — the trend is not a straight line downward. The same survey series found burnout near 87% in 2022, then *substantial improvement* through 2023–2025: job satisfaction more than doubled from its 2022 low, burnout fell markedly, and the share considering leaving dropped significantly. 2026 shows some of that ground being lost again. This is a profession under real strain that has also demonstrated it can recover — which is a different story from terminal decline, and a more honest one.

WHY NURSES LEAVE

The reasons are consistent across the research:

- **Staffing ratios and workload.** Too many patients per nurse is the most-cited driver, and it's the one an individual nurse has least control over.
- **Pay and management.** The top reasons nurses gave for leaving their last position were better pay elsewhere, dissatisfaction with management, and family or personal circumstances.
- **Shift work over decades.** Nights, weekends, holidays, twelve-hour shifts. Tolerable at 23; a different proposition at 45 with a family.
- **Physical toll.** Musculoskeletal injury is common. Backs, knees, feet. This is manual work.
- **Moral injury.** Not the same as being tired — it's the distress of being unable to give the care you believe a patient needs because of time, staffing or system constraints. This is what makes people leave a job they still love.
- **Belonging, in the first year specifically.** Research on new-graduate turnover identifies a low sense of belonging — not being accepted into the team — as a primary driver of first-year departures. That's worth knowing, because it's one of the few factors a student can actively assess when choosing where to work.
- **The gap between why people enter and what the job demands.** Many enter for the caring relationship and find a substantial share of their time going to systems, documentation, and institutional friction. (This is precisely the gap AI may help close — one of the few genuinely hopeful notes here.)

WHAT ACTUALLY HELPS

- Going in with eyes open beats discovering it in year two.
- Specialty and setting matter enormously — community, outpatient, theatre, aged care and school nursing have very different rhythms from acute-ward shift work.
- Programs with broad clinical placements let them find out *which* settings suit them before they're committed.
- **First employer matters more than most families realize.** Nurse residency and structured transition-to-practice programs exist specifically to address first-year turnover, and the belonging factor above is largely a function of the unit they land on. Worth asking about at the job stage, not just the program stage.
- The routes and exits in the next two sections exist precisely because people's lives and bodies change.

Who this work suits — and who it doesn't

Worth being honest about, because fit predicts staying far better than aptitude does. Nursing tends to suit people who:

- **Need to see the point of their work the same day.** Some people can work on abstractions for months; others need to know tonight that today mattered. Nursing is relentlessly concrete.
- **Would rather be moving than sitting.** If a desk feels like a cage, that's real information.
- **Are energised by people rather than drained by them.** The social density of this job is extreme.
- **Get calmer when things go wrong.** This is a genuine and identifiable trait — some people's thinking sharpens in a crisis. It's one of the most valuable things a nurse can have, and it's hard to teach.
- **Like variety more than predictability.** No two shifts are the same. For some that's exhausting; for others it's the entire appeal.
- **Enjoy practical mastery** — hands and knowledge together, not one or the other.
- **Can hold two things at once:** deep care for someone, and the composure to do a difficult thing to them anyway.

AND WHO IT MIGHT NOT

Equally honest:

- People who need **control over their own schedule and workload** will find this hard. That's the single least flexible thing about the job.
- People who need **quiet and solitude to function** will struggle with the social intensity.
- People who **avoid conflict** find the advocacy side difficult — nurses regularly have to push back on doctors, families and systems on a patient's behalf.
- Squeamishness, for what it's worth, is usually *not* a barrier — nearly everyone acclimatises faster than they expect. Conflict-avoidance and needing predictability are the harder ones.

None of these is disqualifying. But someone who recognizes themselves in the second list should think carefully about *setting* — outpatient, community, school nursing, informatics and education offer very different rhythms from acute-ward shift work.

What else is moving this market

The AI score measures task exposure. It cannot see supply, demand, pay, or the ordinary economics of a profession — and those often matter more. For nursing, three forces are acting independently of AI:

A severe and structural shortage. The US is tracking toward a projected deficit of over 250,000 registered nurses by 2028, with roughly 189,100 nursing positions opening each year as older nurses retire. Replacing a single RN costs a hospital around $60,000, which is why retention has become a real institutional priority rather than a slogan. This is the single strongest thing going for the profession, and it has nothing to do with technology.

Training capacity is the bottleneck, not demand. Nursing programs turn away qualified applicants for lack of clinical placements and faculty. That constrains supply further and keeps the graduate market favorable — but it also means entry is more competitive than the shortage headlines suggest.

Conditions, not the calling, are what's under strain. Most nurses would choose the profession again. What has fluctuated is workload, staffing and pay — and those move with health-system funding and politics, not with AI capability. That's worth understanding as a separate axis: this is a career where the technology outlook is stable and the *working conditions* outlook is the volatile part.

The honest summary: the AI score says nursing is protected. The market says nursing is wanted. Neither says the job is easy, and the section above is the counterweight to both.

Sources for this section

- **2026 State of Nursing Survey** — *Nurse.org*. Survey of 2,090 nurses. The single most useful source in this whole profile — it shows the full arc from the 2022 low through recovery to the 2026 softening, and it's the source for both the satisfaction figures and the turnover figures above.
- **The Reality of Nursing Turnover in 2026** — *HealthLeaders, citing Press Ganey's State of Nursing 2026*. Turnover by generation — the Gen Z and Millennial figures above.
- **The Current State of Nursing in 2026** — *American Society of Registered Nurses*. The paradox in one place: rising dissatisfaction alongside high public interest in entering the field and programs turning away qualified applicants for lack of capacity.
- **50% of New Nurses Quit Within 2 Years** — *Nurse.org*. The first-two-years concentration, and the drivers specific to new graduates including the belonging finding.

- **Interventions Promoting Work Engagement and Reducing Turnover of Newly Graduated Nurses** — *Systematic review*. Peer-reviewed, and useful for what actually works — relevant when assessing whether an employer's residency program is substantive.
- **Nursing Burnout: A Profession on the Brink, and the Path Forward** — *Joyce University*. Links staffing to patient outcomes via AHRQ research — useful context for why this is a system problem rather than a personal-resilience one.
- **The Best and Worst Nursing Specialties 2026** — *Nurse.org*. The specialty satisfaction spread. Genuinely useful reading for anyone considering the field.
- **Nurses and NPs share the most and least satisfying aspects of their job** — *The Nursing Beat*. The ranked list of what nurses find rewarding.
- **Nursing Salaries: 2026 Trends, Career Growth & Job Satisfaction** — *Nurse.com*. Pay data and how education and certification shape both earnings and satisfaction.

Several of these are industry or education-sector sources reporting their own surveys, with differing populations. Figures are directional rather than definitive. The consistent findings across all of them — high early-career turnover driven by staffing and workload, alongside strong professional commitment — are well established.

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8. The AI-native advantage — what to actually do about it

This is the section a seventeen-year-old can act on. Everything else describes a landscape; this is about position within it.

The situation, factually: AI is already in clinical settings. As of early 2025 the FDA had authorised 882 AI-enabled medical devices, and industry analysis puts the share of US hospitals using some form of AI-enabled clinical decision support at over 85%. Meanwhile, the 2026 State of Nursing survey reported that AI is arriving in clinical settings *without* the training, trust, or nurse input needed to make it work well.

That gap is the entire opportunity. The technology is deployed. The people who know how to use it well are not. Someone entering nursing now can be among the first cohort who never had to unlearn anything — and that is a real, durable advantage rather than a motivational slogan.

FIRST, A WORD ABOUT HOW SCHOOLS FRAME THIS

Most of what a student hears about AI at school is about cheating. Don't use it for your essay. We can detect it. It's a form of dishonesty.

Those rules exist for a reason, and they're not wrong: using a tool to avoid learning something genuinely costs you the learning. But that framing leaves students with a badly incomplete picture, because it treats AI as a temptation to be resisted rather than a technology to be understood. Those are different things, and only one of them is relevant to a career.

The distinction that matters:

- **Using AI to avoid doing the work** — writing your essay, generating your care plan, skipping the reasoning — leaves you weaker. In nursing this isn't just an academic problem; it's a patient-safety one, because the verification skill in this section only works on top of clinical judgment you actually built yourself.
- **Understanding AI — what it does well, where it fails, and how it behaves in your specific profession** — is a professional competency. It's already written into nursing's own competency frameworks. It's not cheating and it never was.

The schools worrying about the first thing are right to. But nobody is systematically teaching the second, and that's the gap worth stepping into. A student who has spent time deliberately learning *where these systems are confidently wrong in a clinical context* has done something entirely different from a student using them to skip an assignment — and the difference will be obvious to any employer who asks.

Put plainly for the student: don't use AI to do your work. Do use it enough to learn its shape — where it's reliable, where it invents things, where it sounds most convincing at exactly the moment it's wrong. That instinct is the thing you'll be paid for later, and you can only build it by using the tools attentively while the stakes are still low.

FIRST, THE HONEST LEDGER: WHAT AI IMPROVES FOR NURSES

Most of this index describes AI as a threat to careers. In nursing the balance genuinely runs the other way, and the reasons are specific:

It attacks the thing that drives nurses out. The most-cited reason for leaving isn't the patients — it's documentation burden, administrative friction, and never having enough time for actual care. Ambient documentation, automated flowsheet entry and drafted handovers target precisely that. Trials are underway measuring whether this improves nurse wellbeing, not just efficiency.

It gives back the part of the job people came for. If charting drops from a substantial share of a shift to a review-and-correct task, the recovered hours go to the bedside. The mechanism is the same as everywhere else — routine work automates first — but the effect on the person doing the job is very different from professions where the automatable layer was the craft itself.

It supports safety. Continuous monitoring flags deterioration earlier. Decision support catches interaction risks and dosing errors. Predictive tools surface the patient likely to deteriorate before a human would spot it. Nurses catch things; more eyes help.

It creates new senior roles rather than only removing junior ones. Nursing informatics — the bridge between clinical practice and health IT — is growing precisely because implementing these systems requires people who understand both the ward and the software. Very few professions get a new career ladder out of their own disruption.

It may improve the profession's bargaining position. If documentation stops consuming a third of a shift, staffing ratios and workload arguments change shape. That's speculative — but it's a plausible upside worth naming.

The honest caveats: none of this is automatic. Badly implemented systems create alert fatigue instead of relief. Verifying an AI-drafted note is real clinical work, not a freebie. And efficiency gains can be captured as higher patient loads rather than returned as time. The tools are a genuine opportunity; whether the profession gets the benefit depends on implementation — which is an argument for having nurses who understand the technology in the room when those decisions are made.

WHAT AN AI-NATIVE CANDIDATE ACTUALLY LOOKS LIKE

Not someone who has memorised tools. Someone who can do five things:

1. Verify. Knowing when a generated note, alert or triage recommendation doesn't match the patient in front of them. This is a *clinical* skill, not a technical one — it requires knowing the patient well enough to recognize a plausible-sounding error. It is the single most valuable capability in this section, and it is barely taught anywhere.

2. Escalate. Knowing what to do when the output is wrong: who to tell, how to document it, how to override safely. Institutions are still writing these policies. Someone who understands the question is immediately useful.

3. Explain. Being able to tell a patient or family, in plain language, what a system did and why a human is still accountable. Trust is nursing's deepest protection; someone who can articulate it under scrutiny protects it.

4. Recognize the limits. Understanding — at a working level, not an engineering one — the difference between predictive and generative systems, why models produce confident errors, and how algorithmic bias can show up differently across patient populations. This is the ethics dimension and it is increasingly examined in curricula.

5. Contribute. Being the person on the unit who can give useful feedback on whether a system works in practice. Nurses are the largest clinical workforce and the most affected by these tools; the ones who can speak to implementation get pulled into projects, committees and eventually informatics roles.

CONCRETE PREPARATION, BY STAGE

Before starting a program:

- Use the general tools regularly enough to develop instinct for where they're confidently wrong. That instinct is transferable and it's free to build.
- Follow how AI is landing in healthcare specifically — the nursing press covers it continuously.
- When touring programs, ask the AI questions in section 12. Roughly 40% of US nursing programs have updated curricula to include AI and automation topics, which means about 60% have not. That's a real differentiator between programs.

During the degree:

- Treat informatics as a core subject rather than a box to tick. The AACN's Essentials framework already designates informatics and healthcare technologies as a core competency at bachelor's level — the expectation exists; most students treat it as an afterthought.
- In clinical placements, pay attention to which systems the unit actually uses and how experienced nurses work around them. Workarounds tell you where a system fails.
- Practice the verification habit deliberately: when a tool suggests something, ask what would make it wrong before accepting it.
- Consider an informatics elective, certificate or project. Small differentiator now; substantial one at hiring.

Entering the workforce:

- Ask about AI tooling and training at interview. It signals seriousness, and the answer tells you something real about the employer.
- Volunteer for implementation and pilot projects. Units need clinical voices in these and rarely have enough.
- Keep the clinical fundamentals sharp. The verification skill only works on a foundation of genuine clinical judgment — someone who leans on the tools without building

assessment skill has the disadvantage, not the advantage.

WHY THIS IS AN ADVANTAGE RATHER THAN JUST A BURDEN

Every generation entering nursing has had to absorb a technology shift — the transition to electronic records was disruptive and is now unremarkable. What's different here is timing: this cohort arrives while the competency frameworks are still being written and while most working nurses are learning on the job without formal training.

That means a graduate who is genuinely fluent stands out immediately against colleagues with decades more clinical experience — not on nursing skill, which takes years, but on a dimension where seniority confers no advantage. In a profession where advancement usually requires time served, that's an unusual opening.

And it's protective, not just advantageous. Look back at the score table in section 1. The gap between a 2.8 bedside nurse and a 5.4 desk-based one is a gap in *exposure* — and the people most exposed within any profession are the ones whose contribution is the part a system can already do. The nurse who can only follow the protocol is more replaceable than the one who knows when the protocol doesn't fit this patient. AI fluency isn't a separate skill bolted onto nursing; it's part of what keeps someone on the judgment side of that line rather than the executional side.

That's the honest reason to take this seriously. Not because it's exciting, and not because it'll impress anyone — because in every profession in this index, the people who understood the tools early ended up directing them, and the people who didn't ended up competing with them.

The realistic version: this won't make anyone a better nurse than a great experienced one. It will make them useful faster, more visible to the people who staff projects and committees, better positioned for the informatics and leadership paths in section 10, and more firmly on the protected side of their own profession. That's worth having.

SOURCES FOR THIS SECTION

- **AI Competency Gap Emerges in Nursing Education** — *Telehealth.org*. Covers a 2026 review on why pre-licensure informatics competencies need updating, and what the competencies should include — evaluating AI-generated content, ethics and privacy, simulation practice.
- **AI literacy and competency in nursing education** — *Frontiers in Medicine, systematic review and meta-analysis* (2025). Peer-reviewed and global in scope. The serious source on how far AI has actually been integrated into nursing curricula.

- **NLN Vision Statement: AI in Nursing Education** — *National League for Nursing*. What the profession's own body says education should do. Useful for judging whether a program is keeping up.
- **Future Nursing Competencies in an AI-Driven World** — *Nurses Educator*. The FDA device and hospital adoption figures cited above, plus a competency breakdown. Industry source; treat figures as directional.
- **2026 State of Nursing Survey** — *Nurse.org*. (Full link in section 7.) The finding that AI is arriving in clinical settings without adequate training, trust or nurse input — the gap this section is about.

9. The routes in — they're not all the same

This profile has largely assumed a four-year BSN. There are several ways in, with real differences in cost, time and ceiling.

ROUTE	TIME	NOTES
BSN (Bachelor of Science in Nursing)	~4 years	The standard route. Increasingly the baseline many hospitals prefer, and the cleanest path to advanced practice and leadership later.
ADN (Associate Degree in Nursing)	~2 years	Faster and substantially cheaper; qualifies for RN licensure. Many nurses then complete an **RN-to-BSN bridge** while working, often employer-supported. A legitimate route in, not a lesser one — but check local employer preferences, which vary a lot by region.
Accelerated / direct-entry BSN	~12–18 months	For someone who already holds a degree in another field. Intense. Relevant if they're uncertain now and might arrive at nursing later.
LPN/LVN → RN bridge	Staged	Enter the workforce sooner at a lower level, then bridge up, usually while earning. Lowest upfront cost, longest total timeline.
Advanced practice (NP, CRNA, CNM, CNS)	+2–3 years post-BSN	Graduate-level. Higher autonomy and pay; some of these are among the most protected roles in the whole index.

Two things worth checking locally, because they vary: whether area employers effectively require a BSN, and how well-regarded specific bridge programs are with those employers. A quick call to a hospital's nurse recruiter answers both, and that call is worth more than any amount of online research.

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10. Where a nursing degree can take them later

One of the strongest arguments for nursing, and one families rarely consider: it's a qualification with unusually good exits. If they change their mind at 19, or their body changes the calculation at 45, the degree doesn't strand them.

- **Advanced practice** — nurse practitioner, midwife, nurse anaesthetist, clinical nurse specialist. More autonomy, higher pay, strong protection.
- **Specialization** — ICU, emergency, theatre, oncology, pediatrics, mental health. Different work, different pace, same license.
- **Nursing informatics** — the bridge between clinical practice and health IT. Growing fast, and *the people who understand both the ward and the software are exactly who's needed to implement and supervise the AI tools discussed above*. One of the most interesting adjacent paths available right now.
- **Education** — clinical instructor, nurse educator, academic. Persistent shortages here too.
- **Management and leadership** — charge nurse, unit manager, director of nursing. The accountability tier.
- **Public health and policy** — population health, epidemiology, government and NGO roles.
- **Industry** — clinical roles in medical devices, pharmaceuticals, health tech; research and clinical trials coordination.
- **Legal nurse consulting, occupational health, school nursing, aesthetics, travel nursing** — smaller but real, and several offer very different lifestyles.

The important caveat, and it's the same finding as everywhere in this profile: the further a role sits from the bedside, the more its risk score climbs. Informatics, case management and utilization review are genuinely good careers — and they score in the moderate band, not the low one. That's not a reason to avoid them. It's a reason to make the move knowingly.

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11. What to look for in a nursing program

The degree is safe. Not every program is equally good. Score any program against these five.

1. Clinical hours — how many, and how varied? This is the degree's core asset and its structural protection. Ask for the total number of supervised clinical hours and, critically, the *range of settings* — acute, community, mental health, pediatrics, aged care. Breadth here becomes career optionality later.

2. Is AI treated as part of clinical practice? Students should be learning to work with ambient documentation, decision support and triage tools — including **where they fail**. A program that ignores these is training for a job that no longer exists; one that teaches them uncritically is worse. The right posture is supervising and overriding, not trusting.

3. Does it build assessment and judgment, or teach to protocol? Protocol-following is the automatable part. Programs that emphasize clinical reasoning, differential thinking and hands-on assessment are building the part that holds value.

4. NCLEX pass rates and placement outcomes. Ask for the numbers, and ask about the *settings* graduates land in — not just "employed," but where and doing what.

5. Employer relationships and clinical placement quality. Which health systems does the program partner with? Do students compete for placements, or are they guaranteed? Do placements convert to jobs?

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12. Questions to ask an admissions office

Ask these directly. The quality of the hesitation tells you as much as the answer.

On clinical experience:

- How many supervised clinical hours does a typical student complete before graduating?
- Across how many different settings, and are placements guaranteed or competitive?
- Which health systems do you partner with for placements?

On AI and technology:

- How does the curriculum prepare students to work with ambient documentation and clinical decision-support tools?
- Where do students learn to identify when an AI-generated note or alert is wrong?

- Has the curriculum changed in the last two years in response to these tools? What specifically changed?

On outcomes:

- What's your NCLEX first-attempt pass rate, and how has it moved over three years?
- Of last year's graduates, what percentage were working as registered nurses within six months?
- What settings did they go into — and how many are in bedside roles versus desk-based ones?

On attrition — the question most families forget:

- What proportion of students who start the program complete it?
- What are the most common reasons students leave?

Red flags: evasiveness on clinical hours or placement guarantees; "we don't use AI tools here"; outcomes quoted only as "employed" without settings; a curriculum unchanged since 2022; a reluctance to discuss attrition.

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13. Go deeper — further reading

The evidence:

- **Ambient AI's next frontier may be nursing workflow** — *Healthcare IT News*. The most precise piece on exactly where AI is touching nursing: the documentation layer, not the care. Explains why tools built for physician workflows don't simply transfer to nursing.
- **How AI Will Shape the Future of Health Care in 2026** — *SullivanCotter*. Workforce-planning view of where clinical AI lands first — the administrative time sinks — and why radiology is further along than other areas.
- **Barriers and opportunities of scaling ambient AI scribes** — *npj Digital Medicine* (*Nature*). Peer-reviewed, for anyone who wants the primary literature rather than industry commentary.

Worth watching:

- **Ambient AI for Reducing Nursing Staff Documentation Time** — *University of Wisconsin–Madison*, *ClinicalTrials.gov*. A registered clinical trial actively measuring whether ambient AI reduces nursing documentation time and improves wellbeing. Results estimated late 2026 — the kind of evidence that will move this score in a future edition, one way or the other.

- **2026 NSI National Health Care Retention & RN Staffing Report** — *NSI Nursing Solutions*. The primary source for every turnover figure above: 527 hospitals, 40 states, 262,405 registered nurses. The most comprehensive annual workforce dataset in US nursing, and free to read.

- **2026 State of Nursing Survey** — *Nurse.org, 2,090 nurses*. Where the satisfaction, intent-to-leave and specialty figures come from. Read it for the honest bits rather than the headline: the 41% who stay for financial necessity, and the 8% who stay because of good leadership.

- **Job Satisfaction Among Registered Nurses** — *HRSA National Sample Survey of Registered Nurses*. Government data rather than industry survey, useful as a cross-check on specialty-level satisfaction.

The counterargument — read this one:

- **2026 healthcare AI trends: insights from experts** — *Wolters Kluwer.*

> The serious challenge to our low nursing score isn't that AI will replace nurses — nobody credible argues that. It's subtler: **virtual nursing models**, now moving from pilot to real implementation, could restructure *how many* nurses a unit needs and *which* nurses do the bedside work. If a remote nurse can handle admissions, discharge education and documentation across several units, the profession doesn't shrink — it reorganises, and some roles migrate from the bedside to a screen.
>
> That matters, because our entire nursing protection rests on physical presence.
>
> **Where we land:** we score bedside nursing on the work that must happen at a bedside, and that work isn't moving. But this is a genuinely open question rather than a settled one — and it's exactly why we score bedside and desk-based tracks separately. If the virtual tier grows as a *share* of the profession, the average nurse's exposure rises even though the bedside score doesn't. We're watching the ratio, not just the scores, and we'll say so plainly if it shifts.

14. Bottom line

Nursing is among the most protected careers in this index — it holds three of the four most durable protection types, and its entry path is legally guaranteed at a moment when most professions are losing theirs.

The risk question is largely settled. The real questions are the ones families have always faced: the cost of the program against the salary, the physical and emotional demands of the work over decades, and whether the vocation is genuine.

And it's worth restating what the risk analysis can't measure. Most nurses would choose the profession again. What they report finding in it — work that visibly matters the same day, trust at people's hardest moments, mastery of something difficult, and a team worth belonging to — is a combination very few careers offer at all. The hard parts in section 7 are real and shouldn't be minimized. So is that.

What this index adds is one thing: "safe degree" and "safe career" are not the same. The same qualification can lead to a 2.4 or a 5.4 depending on where someone ends up. That divergence tends to appear years later, as people drift toward desk-based roles for entirely reasonable reasons — better hours, less physical strain, career progression.

So the useful advice isn't "do nursing." It's: do nursing, choose a program that delivers broad clinical hours and teaches AI critically, and understand that the protection travels with the bedside. If they move away from direct care later, that may be exactly the right choice for their life — it should just be a decision made knowingly rather than a drift.

Discussion questions

To work through together, after you've both read this.

How to use these: don't do them all at once, and don't treat them as a quiz. Pick a few, answer them both — parent and student — and compare. Several have no right answer; the disagreements are usually the most useful part.

Part 1 — About the work itself

Start here. These are about them, not about AI.

1. What made nursing appealing in the first place? Try to get underneath "helping people" to something specific — a moment, a person, a situation. 2. When you picture them doing this job, what does a Tuesday look like? Compare pictures. They're often very different. 3. Which part of the job do they think they'd like *least*? (This question surfaces more useful information than asking what they'd like most.) 4. Do they want to be around people all day, or does that sound exhausting? Nursing is relentlessly social — that's a feature for some and a real cost for others. 5. How do they feel about the physical side — being on their feet, shift work, bodies, mess, night shifts over decades?

Part 2 — Reacting to the findings

Now bring in what you've read.

6. Was anything in this surprising? Which part, and why? 7. The profile says nursing's protection is at the bedside, not in the badge. Does that change how they think about where they'd want to end up? 8. The desk-based roles — telehealth triage, case management — score nearly twice as high. Those roles usually offer better hours and less physical strain. Is that a trade-off worth making later? Under what circumstances? 9. The profile argues AI is on balance *good news* for nurses, because it removes paperwork and returns time to patients. Does that match what they've heard? Does it change anything? 10. Which of the five "human strengths" do they think they'd be naturally good at? Which would they have to work at?

Part 2b — The harder conversation

Section 7 is the one the score can't tell you. Don't skip it.

10a. Reading the 'what's genuinely hard' part: did any of it surprise them? Did any of it sound like something they'd struggle with, or something they think they could handle?

10b. Shift work for thirty years — nights, weekends, holidays. Have they pictured that concretely, at 25 and at 45? (Neither of you can answer this properly at seventeen. Asking it anyway is the point.)

10c. Moral injury — being unable to give the care you think a patient needs because of staffing or time. How do they think they'd cope with that? This is the thing that makes people leave a job they still love.

10d. Looking at the routes in (section 9): is a four-year BSN definitely right, or is another route worth investigating — and what would each cost in time and money?

10e. Looking at where the degree can lead later — does any exit appeal to them already? Nursing informatics is worth a specific look, given everything else in this profile.

Part 2c — The other side

The 'what's good about it' part of section 7. This matters as much as the risk analysis — probably more.

10f. Of the six things nurses say are most rewarding — making a difference, patient relationships, pride in the work, being good at something hard, the team, variety of settings — which two matter most to them? Which would they miss if the job didn't have it?

10g. Looking at the "who this work suits" list in section 7: which ones sound like them, honestly? Which ones don't? (Parent: answer this about them separately, then compare. The disagreements are the useful part.)

10h. Do they get calmer or more scattered when something goes wrong? Think of an actual example — a crisis, an accident, a moment of pressure. This is one of the best predictors of fitting this work, and it's not something you can train.

10i. Nurse educators report around 68% satisfaction; progressive care around 23% — inside the same profession. Does that change how much weight they put on choosing the *setting* versus choosing the career?

10j. 68% of nurses would choose the profession again, but only 47% would recommend it to someone they love. What do they make of that gap?

Part 2d — The AI question, practically

Section 8 is the one that's actionable. These are about what they'd do, not what might happen to them.

10k. Of the five AI-native capabilities — verify, escalate, explain, recognize limits, contribute — which sounds most interesting to them? Which sounds hardest?

10l. The verification skill (knowing when a system is confidently wrong) only works on top of strong clinical judgment. Does that change how they'd approach the fundamentals in years one and two?

10m. Roughly 40% of US nursing programs have updated their curricula for AI — meaning most haven't. Is that a factor they'd weigh when choosing between programs, and how much?

10n. School AI rules are mostly about cheating. Section 8 argues that's a different thing from understanding AI as a professional skill. Do they see the distinction, and has anyone at school ever framed it that way?

10o. AI is arriving in hospitals faster than the training to use it. Does being early to that feel like an opportunity to them, or like pressure? (Both are legitimate answers, and the answer is useful either way.)

Part 3 — Testing it against reality

The highest-value section. These require going and finding something out.

11. **The most valuable single action in this whole package:** can you find a working nurse — ideally not a family friend who'll be encouraging — for an honest half-hour conversation? Ask them:

> - How did you get in, and would that route still work today? - What's changed about the job in the last three years? - What are the new nurses on your unit doing, and how is that different from when you started? - What do you wish someone had told you before you started?

12. Can you visit a hospital, aged-care setting or clinic — even briefly? Ideas about this job change fast on contact with the real thing. 13. Have they looked at what an actual nursing program's timetable and clinical placement schedule look like? Not the brochure — the real one.

Part 4 — About the program

For when they're shortlisting.

14. Of the programs they're considering, which delivers the most clinical hours, and across the widest range of settings? 15. Ask each program the AI questions from section 12. Compare the answers — the differences will be revealing. 16. What's each program's completion rate, and what do they say about why students leave? 17. If they got into all of them, which would they choose, and what's the actual reason? (Sometimes the honest answer is "my friend's going there" — worth knowing that's the reason.)

Part 5 — Widening the frame

Do these even if they're certain. Especially if they're certain.

18. If nursing weren't available at all, what's the closest thing they'd choose? The answer usually reveals what they're *actually* drawn to. 19. Do they want *nursing*, or do they want what nursing is made of? Someone drawn to it may be drawn to helping people, to science and the body, to high-pressure problem-solving, or to a job with certainty and structure — and those four point to quite different places. Physical therapy, medicine, paramedicine, midwifery and radiography all overlap with nursing but differ in training length, physicality and daily rhythm. 20. Which specialties interest them — ICU, ED, pediatrics, mental health, community, theatre? They don't need to decide now, but the answer shapes which programs suit them. 21. Is there anything

they'd want to pair nursing with — informatics, public health, education, management? Combinations tend to hold value unusually well.

Part 6 — For the parent, alone

Answer these honestly to yourself before the next conversation.

22. Am I hoping they pick this? If so, why — and is that reason about them or about me? 23. What am I actually worried about: that they'll pick the wrong thing, or that they'll struggle, or something else? 24. If they change their mind in two years, what happens? Is that a disaster or a normal part of being nineteen? 25. What's the one thing I most want them to understand from all this — and can I say it in a single sentence, without a number in it?

This profile is one of 16 in the Fall 2026 Degree Risk Index. Every profession is scored against the same six factors with the same weights, and re-scored every six months.

Scores for 2023 and 2025 are retrospectively calculated using the current methodology, not archived from published editions — each year is scored on what was known at the time.

This is a facts document. Where the evidence is contested we've said so, where our own score is challenged by the data we've flagged it, and where we might be wrong we've linked to the people making the opposite case. It is analysis and informed judgment about a fast-moving situation, not a guarantee or a prediction — a starting point for a decision, not the decision itself.

A separate short version of this, written directly to the student, should have come alongside this document. If you don't have it, ask whoever shared this with you.

Technical scoring appendix — Nursing

Pivotum Degree Risk Index — Fall 2026 Edition

This appendix is the audit trail. It sets out the formula, the rating anchors, every factor rating behind every track score in this profile, the backcast arithmetic, a sensitivity analysis, and the specific things that would change our mind.

We publish it because a score nobody can check is an opinion with a decimal point.

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1. The formula

```
Risk = 0.35 × Automatability
     + 0.15 × EntryErosion
     + 0.15 × (10 − Physical)
     + 0.15 × (10 − Trust)
     + 0.10 × (10 − Regulatory)
     + 0.10 × (10 − Judgment)
```

Two exposure factors carry 50% of the weight; three protection factors carry 40%; judgment under novelty carries the remaining 10% and sits on the protection side. The deliberate consequence is that protection can outweigh exposure — which is why a highly automatable job like teaching still scores low.

Bands. Very low below 3.0 · Low 3.0–4.4 · Moderate 4.5–5.4 · Moderate–High 5.5–6.9 · High 7.0 and above.

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2. Rating anchors

HOW THE 0–10 RAW RATINGS ARE ANCHORED

Every factor is rated on its own 0–10 scale before weighting. The anchors are identical across all twenty-eight careers, which is what makes the scores comparable.

Task automatability (A) — *what share of the working week could a capable current system already do to an acceptable standard?*

RATING	ANCHOR
0–2	Almost nothing. The work is physical, relational or wholly novel.
3–4	Administrative surround only — notes, scheduling, correspondence.
5–6	A meaningful minority of the week, mostly documentation and lookup.
7–8	A large share, including tasks central to the junior version of the role.
9–10	Most of the described duties, at or near professional standard.

Entry-path erosion (E) — *how much has the traditional route into this work narrowed?*

RATING	ANCHOR
0–2	Protected by statute or physical necessity. Training hours cannot be automated.
3–4	Stable. Some junior tasks absorbed; the apprenticeship survives.
5–6	Visible narrowing. Employers prefer experience; junior intake reduced.
7–8	Substantial. The work juniors learned on has largely gone.
9–10	The training layer and the automatable layer were the same layer.

Physical / embodied (P) — *must this be done in person, with a body, in an unpredictable setting?*

RATING	ANCHOR
0–2	Fully screen-based. Location-independent.
3–4	Occasional presence; the core work is not physical.
5–6	Regular physical presence, in largely controlled conditions.
7–8	Substantially physical, with some variability of setting.
9–10	Irreducibly physical, in conditions that differ every time.

Human trust / accountability (T) — *does someone need a specific human they can rely on, and must a human answer when it goes wrong?*

RATING	ANCHOR
0–2	Output is anonymous and reviewed by others.
3–4	Work is signed off by someone else. No personal reliance.
5–6	Some client relationship; accountability is institutional.
7–8	Named responsibility, with commercial or professional consequence.
9–10	The relationship is the service, and liability attaches personally.

Regulatory / licensing (R) — *does the law reserve this act to a qualified human*?

RATING	ANCHOR
0–2	No licence exists and none is proposed.
3–4	Certification is customary but not reserved.
5–6	Partial reservation, varying by jurisdiction or task.
7–8	Licensed practice, with some unreserved adjacent work.
9–10	Comprehensive statutory reservation, including of the training route.

Judgment under novelty (J) — *how often does the work present something with no matching precedent*, *where being wrong is costly*?

RATING	ANCHOR
0–2	Routine by design. Deviation is an error, not a case.
3–4	Occasional exceptions, handled by escalation.
5–6	Regular non-standard cases within a known range.
7–8	Frequent genuine novelty with material consequences.
9–10	Novelty is constant, and in some fields adversarially generated.

3. Factor ratings by track

Ratings are the Fall 2026 edition unless a range is shown. **P, T, R and J are held constant across editions** — those protections are structural and do not move on a three-year horizon. All measured movement is in A and E.

BEDSIDE RN (HOSPITAL, ACUTE CARE) — 2.8 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.3	5.0	**6.1**	2.13
Entry-path erosion	15%	1.5	1.8	**2.0**	0.3
Physical / embodied	15%	9.2	9.2	**9.2**	0.12
Human trust / accountability	15%	9.5	9.5	**9.5**	0.07
Regulatory / licensing	10%	9.5	9.5	**9.5**	0.05
Judgment under novelty	10%	8.8	8.8	**8.8**	0.12
				Total	**2.8 → 2.8**

COMMUNITY / HOME HEALTH NURSING — 2.7 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.1	4.6	**5.4**	1.89
Entry-path erosion	15%	1.8	2.0	**2.2**	0.33
Physical / embodied	15%	8.8	8.8	**8.8**	0.18
Human trust / accountability	15%	9.5	9.5	**9.5**	0.07
Regulatory / licensing	10%	9.5	9.5	**9.5**	0.05
Judgment under novelty	10%	8.2	8.2	**8.2**	0.18
				Total	**2.7 → 2.7**

SPECIALIST CLINICAL (ICU, ED, PERI-OP) — 2.4 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	3.4	3.9	**4.8**	1.68
Entry-path erosion	15%	1.8	2.0	**2.0**	0.3
Physical / embodied	15%	9.0	9.0	**9.0**	0.15
Human trust / accountability	15%	9.5	9.5	**9.5**	0.07
Regulatory / licensing	10%	9.5	9.5	**9.5**	0.05
Judgment under novelty	10%	8.5	8.5	**8.5**	0.15
				Total	**2.4 → 2.4**

NURSE PRACTITIONER / ADVANCED PRACTICE — 3.2 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.9	5.7	**6.4**	2.24
Entry-path erosion	15%	2.0	2.2	**2.5**	0.38
Physical / embodied	15%	8.0	8.0	**8.0**	0.3
Human trust / accountability	15%	9.5	9.5	**9.5**	0.07
Regulatory / licensing	10%	9.5	9.5	**9.5**	0.05
Judgment under novelty	10%	8.5	8.5	**8.5**	0.15
				Total	**3.19 → 3.2**

TELEHEALTH TRIAGE — 5.2 (MODERATE)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	3.9	5.7	**6.6**	2.31
Entry-path erosion	15%	3.5	4.0	**4.5**	0.67
Physical / embodied	15%	2.0	2.0	**2.0**	1.2
Human trust / accountability	15%	7.0	7.0	**7.0**	0.45
Regulatory / licensing	10%	9.0	9.0	**9.0**	0.1
Judgment under novelty	10%	5.5	5.5	**5.5**	0.45
				Total	**5.18 → 5.2**

UTILIZATION REVIEW / CASE MANAGEMENT (DESK) — 5.4 (MODERATE)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	3.4	5.1	**6.1**	2.13
Entry-path erosion	15%	4.0	4.5	**5.0**	0.75
Physical / embodied	15%	1.5	1.5	**1.5**	1.27
Human trust / accountability	15%	6.0	6.0	**6.0**	0.6
Regulatory / licensing	10%	8.5	8.5	**8.5**	0.15
Judgment under novelty	10%	5.0	5.0	**5.0**	0.5
				Total	**5.41 → 5.4**

4. Backcast arithmetic

Worked example for the headline track, showing exactly how the three-year movement is composed.

Bedside RN (hospital, acute care)

EDITION	A	E	PROTECTION DEFICIT (FIXED)	SCORE
2023	4.3	1.5	0.37	**2.1**
2025	5.0	1.8	0.37	**2.4**
Fall 2026	6.1	2.0	0.37	**2.8**

Movement decomposition: automatability contributed **+0.63** and entry-path erosion **+0.07**, for a total of **+0.7** points over three years.

The 2023 and 2025 figures are retrospective reconstructions using the current methodology, not archived scores from published editions. They are our best assessment of what each factor would have rated at the time, given what was then demonstrable. They are not measurements.

5. Sensitivity

How far the score moves if a single factor is rated one point differently:

FACTOR	±1 POINT MOVES THE SCORE BY
Task automatability	±0.35
Entry-path erosion	±0.15
Physical / embodied	±0.15
Human trust / accountability	±0.15
Regulatory / licensing	±0.10
Judgment under novelty	±0.10

What this means in practice. A one-point disagreement about automatability moves a score by 0.35 — enough to shift a track between bands at the margins. A one-point disagreement about licensing moves it by 0.10, which almost never changes a band.

So if you disagree with one of our numbers, the automatability rating is where the disagreement matters most, and it is the rating we would most want challenged.

6. Stated limitations

We measure capability, not deployment. Scores reflect exposure to what AI can already do at the leading edge — not how much any particular employer has adopted. Adoption lags capability by years and lags unevenly: large, well-funded organisations first; small, rural, public-sector and thin-margin employers much later. The lag is a buffer, not a shelter. It buys time without changing direction.

We do not measure demand. A task can be highly automatable and highly in demand simultaneously. Where the labour market and our framework disagree, we say so in the profile rather than adjusting the score to fit.

We do not measure oversupply. The number of people entering a field is outside our six factors, and for some careers it matters more than automation.

We do not measure industry economics. A profession can contract for reasons entirely unrelated to technology.

We do not measure whether the work is worth doing. Pay, satisfaction, debt, hours and meaning sit outside the score and are covered separately in each profile — often at greater length, because for several careers they matter more.

Sub-track definitions are ours. Job titles vary between employers and countries. We have chosen tracks that reflect genuinely different work, not official classifications.

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7. What would change our mind

Each edition names specific, checkable indicators. If these move, the scores move — including downward.

Across the index:

- **Graduate hiring share.** If employers in the professions where we rate entry-path erosion highly return to hiring juniors at pre-2023 rates, those ratings fall.
- **Content of mandated training hours.** Several professions protect their on-ramp through required supervised experience. If the content of those hours shifts from producing work to supervising it, the protection weakens without any rule changing.
- **Offsite and prefabricated construction share.** The clearest test of our physical-protection ratings: automation performs well in controlled settings, so moving work indoors raises exposure without any robotics breakthrough.

- **Verified deployment rather than capability.** If adoption stalls materially below capability for several editions, our leading-indicator approach is overstating near-term risk and we will say so.

We will report movement in either direction. A framework that only ever revises upward is not measuring anything.

Scores measure exposure to what AI can already do — not how much any particular employer has deployed. The 2023 and 2025 figures are retrospective reconstructions using the current methodology.